Adjuvant trastuzumab emtansine (T-DM1)

NCCP 00659 · Draft — independent clinical, pharmacy and patient/PPI review pending

PRACTICAL TREATMENT GUIDE

Your practical treatment plan

T-DM1 by IV infusion every 21 days for 14 postoperative cycles, unless recurrence or unacceptable toxicity requires a change.

STEP 1

Before the first visit

Blood tests, cardiac/immune checks as relevant, medicines review and the consent conversation come first.

STEP 2

Plan venous access

A cannula, PICC or implanted port may be used. If a port is chosen, a trained team inserts it in a separate procedure with local anaesthetic and sometimes sedation; the team explains fasting, blood-thinner and wound instructions.

STEP 3

Infusion day

Trained staff check and access the line, give the prescribed premedicines and infusions, and monitor for reactions. Tell the nurse immediately about pain, burning, rash or breathlessness.

STEP 4

Home and next review

This regimen does not routinely need a take-home infusion pump. Staff disconnect the infusion; a port is de-accessed when appropriate. Keep line-care and next-visit instructions.

Port insertion and removal

The line team explains placement, dressing care and when normal activities can resume. A port is accessed with a special needle during treatment. Removal of the implanted device is a separate planned procedure when no longer needed; do not remove or adjust it yourself.

PICC care and urgent line symptoms

A PICC usually needs regular dressing and flushing appointments even between treatments. Fever, redness, swelling, pain, leakage or damage requires prompt advice. Your hospital supplies the exact care instructions.

Visit time and transport

First visits can take longer because of loading doses, premedicines and observation. Ask the unit about expected time, transport and any escort needed after sedation or line insertion.

Sources: NCCP 00659 v2 · official regimen · eviQ · implanted port patient information · eviQ · PICC patient information. Draft — independent clinical, pharmacy and patient/PPI review pending.